



Neurodiversity, identity, and hypostatic abstraction

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Accepted: 22 March 2025 / Published online: 10 April 2025
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Abstract

The Neurodiversity (ND) movement demands that some psychiatric categories be de-pathologized. It has faced much criticism, leading some to despair whether it can ever be brought together with psychiatry. In this paper, we argue for a particular understanding of this central demand of the ND movement. We argue that the demand for de-pathologizing is the rejection of (paradigmatically) autism as a *hypostatic abstraction*; the ND movement is committed, first and foremost, to the reconceptualization of autism not as something one *has*, but as something one *is*. We distinguish between two senses of autistic identity—one pre-reflective, and one social and political—operative in this reconceptualization. This understanding of the ND movement is centrally about a rethinking of the *relation* between a subject and a psychiatric label. It is not about reconceptualizing psychiatric categories in terms of advantageous variations, as we believe critics fear. Our understanding of what the ND movement is asking for has the noteworthy consequence that many of the most influential criticisms of the ND movement are missing the mark and worries about the impossibility of reconciling the movement with psychiatry are unwarranted.

Keywords Neurodiversity · Psychiatric categories · Theories of psychiatry · Biomedical model · Hypostatic abstraction · Anti-psychiatry

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神经多样性、身份与实体抽象

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收稿日期: 2025年3月22日 / 在线发布日期: 2025年4月10日
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摘要

神经多样性运动要求对一些精神病学分类进行去病理化。它面临诸多批评, 导致一些人对其能否与精神病学相容感到绝望。在本文中, 我们论证了对神经多样性运动这一核心诉求的一种特定理解。我们认为, 去病理化的诉求是拒绝将(典型意义上的)孤独谱系视为一种实体抽象; 神经多样性运动首要致力于将孤独谱系重新概念化为一个人所是的东西, 而非一个人拥有的东西。我们区分了在此重新概念化过程中起作用的两种孤独谱系身份——一种是前反思的, 另一种是社会与政治的。对神经多样性运动的这种理解, 核心在于重新思考主体与精神诊断标签之间的关系。它并非如批评者所担忧的那样, 将精神病学分类重新概念化为有利的变异。我们对神经多样性运动诉求的理解, 产生了一个值得注意的后果: 许多最具影响力的对神经多样性运动的批评都未能切中要害, 而关于该运动无法与精神病学调和的担忧也是没有根据的。

关键词 神经多样性 · 精神病学分类 · 精神病学理论 · 生物医学模型 · 实体抽象 · 反精神病学

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1 Introduction

Activists in the neurodiversity (ND) movement make a central demand: that certain categories—e.g., autism, ADHD, dyspraxia, Tourette syndrome, etc.—be de-pathologized. They think that instead of being considered disorders, such categories should be considered neurodivergences, and in some cases social identities. (In this paper, we focus on autism.) Ultimately, this demand is political. It is meant to be, or perhaps to support, a call for recognition, inclusion, and de-stigmatization. But what does this call to ‘de-pathologize’ *mean* when uttered by members of the ND movement? Most criticisms of the ND movement have expressed worries about these calls for inclusivity and recognition. In this paper, we argue that the best understanding of the core ND demand involves reconceptualizing neurodivergence as identity-making, rejecting what we call the *hypostatic abstraction principle* (HAP). Understanding the movement in this way reveals major criticisms of the movement to be founded on misapprehensions.

HAP says psychiatric diagnoses identify entities which are separable from their subjects, and which ought to be the goal of psychiatry to eliminate. HAP makes use of the notion of a hypostatic abstraction, a notion also put to use by Rashed (2020), and which is originally owed to C. S. Peirce. We begin by explicating this notion and making the case that the ND movement is best understood as rejecting HAP.

We claim that the central ND commitment is to an understanding of certain psychiatric categories as identity-making and self-constitutive. In other words, what underlies the rejection of psychiatric diagnoses as hypostatic abstractions, for ND activists, is simply the claim that being autistic is part of what one *is* in such a way that it *cannot* or *should not* be separated from one. Hence, the reasoning goes, HAP should be rejected. Importantly, rejecting HAP does not amount to rejecting the reality of psychiatric conditions or the usefulness of psychiatric labels altogether. Rather, it involves challenging the specific idea that such conditions are best conceptualized as separable entities, distinct from the lived realities of the people who embody them.

One consequence of understanding the ND movement in the way we suggest is that influential critics of it seem to be missing their target. Critics often misinterpret the ND movement as being committed to particular claims about the empirical or metaphysical nature of certain psychiatric categories. This can seem an intuitive way of reading the movement, in part, because of the intellectual context provided by anti-psychiatry, a shadow from which much discussion of controversial psychiatric categories has never fully emerged. In the following discussion we draw connections between the legacy of anti-psychiatry and hypostatic abstraction, especially during our exposition of that key latter notion.

We argue that the intent of ND activists is not to argue for the adoption of an empirical or metaphysical conceptualization of psychiatric categories that necessarily differs from that used and advocated for by the psychiatric establishment, but to rethink the *relationships* which obtain between people and these categories. Critics have either not fully appreciated this fact or are implicitly committed to HAP themselves. Given that ND activists appear to be putting pressure directly on HAP, recognizing that this is at the core of the discussion is necessary for the discussion to advance productively.

1 引言

神经多样性运动的行动者们提出了一项核心诉求：某些分类——例如孤独谱系、注意缺陷多动障碍、运动协调障碍、图雷特综合征等——应当被去病理化。他们认为，这些分类不应被视为障碍，而应被视为神经特殊性，在某些情况下甚至应被视为社会身份。（本文重点关注孤独谱系。）归根结底，这一诉求是政治性的。它旨在成为——或旨在支持——一种对承认、包容和去污名化的呼吁。但当神经多样性运动的成员发出“去病理化”的呼吁时，这究竟意味着什么？大多数对神经多样性运动的批评都表达了对这些包容与承认诉求的担忧。在本文中，我们认为，对神经多样性运动核心诉求的最佳理解，在于将神经特殊性重新概念化为身份构建，并摒弃我们所谓的实体抽象原则。以此种方式理解该运动，便可揭示出针对该运动的主要批评乃是基于误解。

HAP认为精神病学诊断所识别出的实体，是可与主体分离的，并且应是精神病学力求消除的目标。HAP运用了实体抽象这一概念，Rashed (2020) 也使用了这一概念，而它最初源于C. S. Peirce。我们首先阐释这一概念，并论证神经多样性运动最好被理解为对HAP的拒绝。

我们认为，神经多样性运动的核心主张在于，将某些精神病学范畴理解为身份塑造和自我构成的。换言之，对于神经多样性活动家而言，拒绝将精神病学诊断视为实体抽象的深层原因很简单，即认为孤独谱系特质是一个人之所以为其自身的一部分，以至于它无法或不应被剥离。因此，推理得出，HAP应被拒绝。重要的是，拒绝HAP并不意味着否定精神病学状况的现实性或精神诊断标签的整体有用性。相反，它挑战的是这样一种特定观念，即这些状况最好被概念化为可分离的实体，独立于体现它们的个人的生活现实。

按照我们建议的方式来理解神经多样性运动，会带来一个后果：那些有影响力的批评者似乎并未击中目标。批评者常常误以为神经多样性运动致力于主张某些精神病学分类在经验或形而上学层面的特定本质。这似乎是一种直观的解读方式，部分原因在于反精神病学所提供的智识背景——关于有争议的精神病学分类的许多讨论，至今仍未完全摆脱其阴影。在接下来的讨论中，我们将把反精神病学的遗产与实体抽象联系起来，尤其是在阐述后一个关键概念时。

我们认为，神经多样性活动家的意图并非主张采用一种必然不同于精神病学体系所使用和倡导的、关于精神病学分类的实证主义或形而上学概念化，而是重新思考人与这些分类之间存在的关系。批评者要么未能充分理解这一事实，要么自身就隐晦地认同HAP。鉴于神经多样性活动家似乎正直接对HAP施压，认识到这一点是讨论的核心，对于推动讨论富有成效地进展是必要的。

2 The core demand of the ND movement

2.1 The hypostatic abstraction principle (HAP)

Our proposal is that the central demand of the ND movement is best understood in terms we borrow from Rashed (2020). Rashed, in turn, makes use of a notion first formulated by Peirce, viz., that of *hypostatic abstraction*. Because this notion is key to our discussion, it is worth discussing it in some detail here.

As J. Jay Zeman (1981) points out, Peirce was keen to distinguish two kinds of abstraction. Zeman provides the following reference from a 1904 letter (213):

There are two entirely different things that are often confused from no cause that I can see except that the words abstract and abstraction are applied to both. One is αφαιρεσις leaving something out of account in order to attend to something else. That is precise abstraction. The other consists in making a subject out of a predicate. Instead of saying, Opium puts people to sleep, you say it has dormitive virtue. This is an important proceeding in mathematics. For example, take all “symbolic” methods, in which operations are operated upon. This may be called subjectal abstractions.

As Zeman notes, “Peirce also (and more frequently) calls that [latter, “subjectal”] process ‘hypostatic abstraction.’” (Ibid.) Bracketing complications brought in by the peculiarities of Peirce’s logic, hypostatic abstraction in general can be understood as a mathematical operation on a propositional formula that increases the arity of the main predicate by 1 and translates the abstracted former predicate term into one of the new argument positions. Thus, before hypostatic abstraction, the one-place predicate P could be said to hold of an individual a ; after abstraction some two-place relation R is said to hold between an entity, p -ness, and a : $R(p, a)$. Before abstraction: Janie is tall. After abstraction: tallness is a thing Janie stands in some relation (say, the relation of having) to. Hypostatic abstraction is the process by which, in this case, tallness is, we might say, reified, reframed as distinct entity in its own right.¹

Hypostatic abstraction is key to Peirce’s understanding of mathematics. It is not difficult to see why this is. For instance, it is thinking not just that a curve has the property of being sloped to some degree, but thinking that the slope itself can be formalized as a kind of mathematical subject in its own right that allows it to be investigated mathematically and revealed to have interesting properties of its own, some of which, in turn, may be subject to hypostatic abstraction themselves (as when the first derivative of a line is seen to itself have a slope and we get the original curve’s second derivative.)

Rashed’s focus is Mad activism. How does this relate to Peirce’s notion of hypostatic abstraction? Mad activism is a set of movements which, among other things, reject the medicalization of madness. Rashed thinks that for psychiatry to meet the

¹ There are contexts, of course, where such reification is plainly fallacious. The unrestricted use of hypostatic abstraction may or may not fare well against the sort of ontological austerity which is the hallmark of much analytic metaphysics. But we set that issue aside.

2 神经多样性运动的核心诉求

2.1 实体抽象原则 (HAP)

我们的提议是，神经多样性运动的核心诉求，最好借用Rashed (2020) 的术语来理解。而Rashed则运用了一个最初由Peirce提出的概念，即实体抽象。由于这个概念是我们讨论的关键，值得在此稍作详细探讨。

正如 J. Jay Zeman (1981) 所指出的，Peirce 热衷于区分两种抽象。Zeman 引用了 1904 年一封信件中的以下论述 (第 213 页)：

有两种完全不同的事物常常被混淆，在我看来，除了‘抽象’这个词被同时用于两者之外，别无他因。一种是 αφαιρεσις 为了关注某物而将另一物排除在考虑之外。这被称为精确抽象。另一种则是将谓词转变为主体。例如，不说‘鸦片使人入睡’，而说它具有‘催眠效力’。这在数学中是一个重要的过程。例如，所有‘符号化’方法，其中运算本身被进行运算。这可称为主体抽象。

正如 Zeman 所指出的，“Peirce 也（并且更频繁地）将那个[后一种‘主体’]过程称为‘实体抽象’。”（同上）暂且不论 Peirce 逻辑的特殊性所带来的复杂细节，实体抽象总体上可以理解是对命题公式的一种数学运算，它将主谓词的元数增加 1，并将被抽象出的原谓词项转换为新参数位置之一。因此，在实体抽象之前，一元谓词 P 可以说适用于个体 a ；抽象之后，某个二元关系 R 则被说成存在于一个实体 p -ness 与 a 之间： $R(p, a)$ 。抽象前：珍妮是高的。抽象后：高是珍妮与之处于某种关系（例如，拥有关系）的一个事物。实体抽象就是这样一个过程，在此例中，高被，我们或许可以说，物化了，被重新构建为一个独立的实体。¹

实体抽象是Peirce理解数学的关键。这并不难理解。例如，它不仅仅意味着认为曲线具有某种程度的斜率属性，而是认为斜率本身可以被形式化为一种独立的数学主体，从而允许对其进行数学研究，并揭示其自身具有的有趣特性——其中一些特性本身又可能成为实体抽象的对象（就像一条线的一阶导数本身被视为具有斜率，从而我们得到原始曲线的二阶导数）。

Rashed 关注的是疯狂行动主义。这与 Peirce 的实体抽象概念有何关联？疯狂行动主义是一系列运动，其主张之一便是拒绝疯狂的医学化。Rashed 认为，精神病学若要满足

¹ 当然，在某些语境中，这种具体化显然是谬误的。实体抽象的无限制使用，在应对那种作为许多分析形而上学标志的本体论节俭时，其效果可能好也可能差。但我们将这个问题暂且搁置。

demands of Mad activists, it needs to abandon what he calls, fittingly, ‘the hypostatic abstraction’ (Rashed, 2020), which for him is the conception of the object of medical intervention as a *thing*, separate from its subject, which should be identified, treated, and, if possible, removed. Evidently, Rashed’s claim that psychiatry can only meet the demands of Mad activism by rejecting the hypostatic abstraction involves thinking that Mad activists *want* it abandoned. We argue that the same should be said about the ND movement.

Rashed’s use of the notion of a hypostatic abstraction clearly imports much that is not present in Peirce. Rashed is talking about a (perhaps often unacknowledged) theoretical framework; Peirce is talking about something of a fundamental logical or cognitive operation; for Peirce, hypostatic abstraction is domain-general and plays a special role in mathematics, whereas Rashed is focused on how it operates in medicine. Rashed is also focused, in the first instance, on a certain product of hypostatic abstraction, not the process.

Perhaps most importantly, Rashed understands medicine’s commitment to hypostatic abstraction as involving a normative component: not only is it that the objects of medical intervention are entities separable from their subjects, they *ought to be separated from their subjects*.

Consider, then, the following formulation of what Rashed takes to regulate medicalized psychiatry:

The Hypostatic Abstraction Principle (HAP): Psychiatric categories are hypostatic abstractions, i.e., they pick out things which are separable from those persons who are their subjects and which it is the goal of medicine to separate from those subjects.

According to Rashed, Mad activism is committed to a denial of HAP for a large range of psychiatric categories. For instance, Mad activists prefer to say that someone is schizophrenic, rather than *having* schizophrenia. Because HAP has one component which concerns how psychiatric categories should be conceived and another which concerns how they should be treated, one could deny HAP by denying either. Taking our cue from Rashed, we think that Mad activists are best understood as rejecting HAP by denying both. We also think ND activists should be understood in the same way.

We should say a little more about what we mean by the ‘regulation’ of medicalized psychiatry by HAP. Such regulation need not involve the explicit cognitive endorsement of HAP by individual psychiatrists. As we will see below, many of the critics of the ND movement we discuss seem to be logically committed to the truth of HAP by the very structure of their criticisms, but the mere fact that HAP is operative in determining the shape of psychiatric practice does not require (though it is of course consistent with) any particular explicit cognitive attitudes towards HAP on the part of its practitioners. Indeed, we take Rashed to be *excavating* HAP, such that it may well be surprising to many in the psychiatric community. What the regulation of psychia-

trian rights activists’诉求,就必须放弃他所称的、恰如其分的‘实体抽象’(Rashed, 2020)。在他看来,这种抽象是将医学干预的对象构想为一个事物,独立于其主体,需要被识别、治疗、并在可能的情况下被移除。显然,Rashed关于精神病学只有拒绝实体抽象才能满足疯狂行动主义诉求的主张,蕴含了疯癫权益行动者希望放弃它的想法。我们认为,对于神经多样性运动,也应作如是观。

Rashed 对实体抽象概念的使用显然引入了许多皮尔士思想中不存在的内容。Rashed 谈论的是一个(或许常被忽视的)理论框架;而皮尔士谈论的是一种基本的逻辑或认知操作;对皮尔士而言,实体抽象是领域通用的,并在数学中扮演特殊角色,而 Rashed 则专注于它在医学中的运作方式。此外,Rashed 首先关注的是实体抽象的某种产物,而非过程本身。

或许最重要的是,Rashed 将医学对实体抽象的承诺理解为包含了一种规范性成分:不仅医学干预的对象是可与其主体分离的实体,而且它们应当与其主体分离。

那么,请考虑以下关于 Rashed 认为规范医学化精神病学的表述:

实体抽象原则(HAP):精神病学分类是实体抽象,即它们挑选出那些可与其主体(即承载这些分类的人)分离的事物,而医学的目标正是将这些事物从其主体中分离出来。

根据Rashed的观点,疯狂行动主义致力于否认针对广泛精神病学分类的HAP。例如,疯癫权益行动者更倾向于说某人是精神分裂症患者,而不是患有精神分裂症。因为HAP包含一个关于精神病学分类应如何被理解的组成部分,以及另一个关于它们应如何被对待的组成部分,人们可以通过否认其中任何一个来否认HAP。借鉴Rashed的提示,我们认为疯癫权益行动者最好被理解为通过同时否认两者来拒绝HAP。我们也认为神经多样性活动家应以同样的方式被理解。

关于HAP对医学化的精神病学的‘规制’,我们需要再多说几句。这种规制并不要求个体精神病医生明确地在认知上赞同HAP。正如我们将在下文看到的,我们所讨论的许多神经多样性运动的批评者,似乎从其批评的内在逻辑结构上就已经在逻辑上承诺了HAP的真实性,但HAP在决定精神病学实践形态中发挥作用这一事实本身,并不要求(尽管当然也不排斥)其从业者对HAP持有任何特定的、明确的认知态度。事实上,我们认为Rashed正在发掘HAP,这很可能会让精神病学社群的许多人感到惊讶。精神病学的规制所

try requires is simply that psychiatrists engage in hypostatic abstraction as a way of achieving what they take to be their professional aims.²

What sense of ‘separability’ is invoked by HAP? It is clear enough that in this context, the relevant separability, which must be something that could actually be achieved by medical intervention, at least in principle, and which would generally be medically desirable to actualize, is cure by elimination. Of course, there are many medical contexts where cure is not a realistic goal, but insofar as treatment of some kind is practicable and cure is the limit case of treatment, cure is in a derivative sense still desirable.

Mad activists deny both components of HAP. They argue that a different relation obtains between Mad people and their psychiatric diagnoses than that presupposed by HAP. According to them, such diagnoses correspond to features of such persons’ *identities*. We think it is the rejection of HAP that also underlies the central ND demand for de-pathologization and we think that it works the same way: ND activists are arguing that autism is not separable from them, i.e., that it is, to some ineliminable degree, a component of their identities. This suggests that autism is not merely a set of impairments to be treated or eliminated, but a fundamental part of how autistic people experience the world, engage with others, and construct their sense of self.

2.2 Two senses of autistic identity

What is the relevant sense of identity which is doing all of the heavy lifting against HAP for Mad activism and the ND movement? Consider the following passage from Sinclair, who first articulated that idea for autism in an essay based on a presentation given at the 1993 International Conference on Autism in Toronto:

Autism isn’t something a person has, or a ‘shell’ that a person is trapped inside. There’s no normal child hidden behind the autism. Autism is a way of being. It is pervasive; it colours every existence. It is not possible to separate the autism from the person – and if it were possible, the person you’d have left would not be the same person you started with. (Sinclair, 1993).

Several decades later, we see the following: “Autism is also celebrated as an inseparable aspect of identity (Kapp et al., 2013; Bumiller, 2008), demanding ‘recognition and acceptance’ (Jaarsma & Welin, 2012)” (Rosqvits et al. 2020, p.6). Another prominent example:

[A]utism is not best thought of as a condition that alters a preexisting person, but rather as a different ‘way of being’. Hence, in contrast to speaking of ‘people with autism’ or ‘people with schizophrenia’, neurodiversity paradigm proponents talk of ‘autistic people’ or ‘schizophrenic people’. This framework allows for categories such as ‘autism’, ‘schizophrenia’ and so on, for which no biological essence has been found (despite decades of searching), to be under-

要求的，仅仅是精神病医生将实体抽象作为一种手段，以实现他们自认的职业目标。²

HAP 援引了何种意义上的‘可分离性’？很明显，在此语境下，相关的可分离性必须是至少原则上可通过医学干预实际达成的，并且通常实现它在医学上是可取的，这种可分离性即消除式治愈。当然，在许多医学情境中，治愈并非一个现实的目标，但只要某种治疗是可行的，而治愈是治疗的极限情况，那么治愈在衍生意义上仍然是可取的。

疯癫权益行动者否认 HAP 的两个组成部分。他们认为，Mad 群体与其精神病学诊断之间的关系，不同于 HAP 所预设的那种关系。据他们所言，此类诊断对应的是这些人身份的特征。我们认为，对 HAP 的拒绝同样构成了神经多样性核心诉求——去病理化——的基础，并且其运作方式相同：神经多样性活动家主张孤独谱系与他们是不可分离的，也就是说，它在某种程度上是他们身份中一个无法消除的组成部分。这表明孤独谱系不仅仅是一系列有待治疗或消除的障碍，更是孤独谱系群体体验世界、与他人互动以及构建自我认知的根本部分。

2.2 孤独谱系身份的两种含义

对于疯狂行动主义和神经多样性运动而言，是哪种相关的身份概念在与 HAP 进行着艰苦卓绝的抗争？请思考以下摘自 Sinclair 的段落，他于 1993 年在多伦多举行的国际孤独谱系大会上发表演讲，并在一篇论文中首次阐述了这一关于孤独谱系的观点：

孤独谱系并非一个人所拥有的东西，也不是将人困于其中的‘外壳’。并不存在一个隐藏在孤独谱系背后的正常孩子。孤独谱系是一种存在方式。它是渗透性的；它影响着存在的方方面面。将孤独谱系与这个人分离开来是不可能的——即便可以分离，那么剩下的那个人也已不再是原先的那个人了。（Sinclair, 1993）。

数十年后，我们看到了这样的表述：“孤独谱系也被颂扬为身份不可分割的一部分（Kapp 等, 2013; Bumiller, 2008），要求‘认可与接纳’（Jaarsma 与 Welin, 2012）”（Rosqvits 等, 2020, 第6页）。另一个突出的例子是：

[A]孤独谱系不应被视为一种改变先存个体的条件，而应被看作一种不同的“存在方式”。因此，与谈论“患有孤独谱系的人”或“患有精神分裂症的人”不同，神经多样性范式的支持者谈论的是“孤独谱系群体”或“精神分裂症群体”。这一框架允许诸如“孤独谱系”、“精神分裂症”等类别存在——尽管经过数十年的探索，尚未发现其生物学本质——被视为

² We would like to thank an anonymous reviewer for raising the importance of distinction between endorsing a principle such as HAP and merely engaging in hypostatic abstraction.

² 我们要感谢一位匿名审稿人，他指出了区分赞同 HAP 这样的原则与仅仅进行实体抽象之间的重要性。

stood as minorities in the way that ethnic minorities are considered meaningful (i.e., despite not being real in a biological sense). That is, they share a similar way of being, which is meaningful due to complex historical circumstances. It also opens the room for talking about, say, autism, as a political identity, with its own culture and shared vocabulary. (Chapman, 2019, p.375).

Two different threads are apparent from these passages. First, we find the idea that, for autistic people, autism is part of what we might call the basic pre-reflective structure of experience. It is not an alteration to a more basic non-autistic way of experiencing the world (which is not to say it is *incomparable* with non-autistic experience.) And such a structure is ‘pervasive’ in the sense that it is basic, it ‘colours every existence’ by structuring fundamental features of one’s lived experience rather than being something one adopts voluntarily or by way of commitment or valuation. Such a pre-reflective structuring is akin to the Husserlian notion of “pre-reflective self-awareness” that Zahavi describes as a

pervasive self-awareness, which by no means is to be understood as a particular intentional act but as a dimension of basic self-manifestation that precedes and founds reflection” (Zahavi, 1999, p.62).

Understanding autism to play such a role determining the basic pre-reflective structure of experience contrasts with thinking of autism, in the first instance, as an experiential or intentional object. One does not, in this mode, direct one’s mind *at* one’s autism. Rather, it is a foundational aspect of experience and as such constitutes a fundamental aspect of the self, an aspect through which, in the first instance, other things show up for one and are conceived. In part, this structuring is properly thought to involve basic modes of embodiment, as emphasized by Boldsen who, in her phenomenological account of autism subjectivity, emphasizes how autism affects the pre-reflective, embodied aspects of experience. She argues that autistic people often experience the world differently at a fundamental, bodily level, which shapes their interactions and perceptions before any conscious reflection (Boldsen, 2018).

Such pre-reflective structuring of experience can be leveraged into a social or political identity through group identification. Autism, of course, is a social category, and in the sense of social identification, there would be nothing to identify *with* if this were not the case³. However, because self-identifying as autistic is intended to signify something pre-social—something that autistic people share by virtue of having a different pre-reflectively structured experience than non-autistics—it would be misleading to say that autism, as an identity, is *merely* a social identity.

This is in part because social identities are, at least to a certain extent, a matter of how one is identified by others. The kind of identity we think is relevant to ND activists concerns, at least in the first instance, one’s self-understanding, one’s way of picking out and identifying *oneself*. Yet, even this is potentially misleading. While

³ The ND movement also explicitly recognizes it as political as it takes it “to indicate a marginalized minority rather than a medical condition, making it more in line with political identities such as ‘trans’ or ‘gay’ than something that we should attempt to treat or cure” (Chapman, 2020, p.800).

族裔少数群体那样有意义的少数群体（即，尽管在生物学意义上并非真实存在）。也就是说，他们共享一种相似的存在方式，这种相似性在复杂的历史背景下具有意义。这也为探讨，例如，将孤独谱系，视为一种政治身份，并拥有其自身的文化和共享词汇，打开了空间。(Chapman, 2019, p.375).

从这些段落中可以明显看出两种不同的线索。首先，我们发现这样一种观点：对于孤独谱系群体而言，孤独谱系是我们可能称之为经验的前反思基本结构的一部分。它并非对一种更基本的非孤独谱系体验世界方式的改变（这并非说它与非孤独谱系体验是不可比较的）。并且，这种结构是‘普遍存在’的，其意义在于它是基本的，它通过构建个体生活经验的基本特征来‘浸染每一种存在’，而非个体自愿或通过承诺或价值评判所采纳的东西。这种前反思的结构类似于胡塞尔概念中的“前反思自我意识”，扎哈维将其描述为一种

普遍存在的自我意识，这绝不能被理解为一种特定的意向性行为，而应被理解为一种基本的自我显现维度，它先于反思并为反思奠定基础”（扎哈维，1999，第62页）。

理解孤独谱系能起到如此作用，决定经验的基本前反思结构，这与首先将孤独谱系视为一个经验性或意向性对象形成对比。在这种模式下，人并非将心智指向自身的孤独谱系。相反，它是经验的一个基础方面，并因此构成了自我的一个根于本方面，一个首先通过它，其他事物才为人显现并被理解的方面。在某种程度上，这种结构恰当地被认为涉及基本的具身模式，正如Boldsen所强调的那样，她在对孤独谱系主体性的现象学描述中，强调了孤独谱系如何影响经验的前反思、具身方面。她认为，孤独谱系群体通常在根本的、身体的层面上以不同方式体验世界，这塑造了他们在任何有意识反思之前的互动和感知（Boldsen, 2018）。

这种经验的前反思结构可以通过群体认同被利用，形成一种社会或政治身份。当然，孤独谱系是一个社会范畴，在社会认同的意义上，如果情况不是这样，就没什么可认同的³。然而，因为自我认同为孤独谱系旨在标示某种前社会性的东西——某种孤独谱系群体因拥有与非孤独谱系者不同的前反思结构化经验而共享的东西——所以说孤独谱系作为一种身份，仅仅是一种社会身份，将会产生误导。

这部分是因为社会认同至少在某种程度上取决于一个人如何被他人识别。我们认为与神经多样性活动家密切相关的身份，至少在最初意义上，关乎个人的自我理解，即一个人挑选和识别自身的方式。然而，即使这一点也可能具有误导性。尽管

³ 神经多样性运动也明确地将其视为一种政治立场，因为它认为其“代表的是一个被边缘化的少数群体，而非一种医学状况，这使其更接近于‘跨性别’或‘同性恋’这类政治身份认同，而非我们应该试图治疗或治愈的某种病症”（Chapman, 2020，第800页）。

social and political identification involves an identificatory *act*, the basic pre-reflective structuring of experience need not. That is, unlike social identification, the pre-reflective structuring of experience need not be constituted by the performance of any reflective acts of judgment in which some social property is predicated of the self.

We should carefully distinguish, then, the pre-reflective structuring of experience that is found in autism from social or political identification as one whose experience is so structured. The former is an identity in the sense that it is essential to who one is; the latter is an identity in the sense of being a social identity that one reflexively applies and through which one's place in the social world is (partially) assigned. As with all social identities, belonging is normatively regulated in the sense that members are expected to conform their beliefs and behavior to group-specific norms. But because the grounds for identification as autistic are first-personal to an unusually high degree for social kinds, those grounds take on their own independent significance. And since those grounds are not themselves social, we are forced to acknowledge that as an identity, autism has these two different elements: the pre-reflective structuring of experience that is found in autism, and social or political identification; and neither can be reduced to the other. Moreover, the acquisition of the various cognitive capacities required to identify oneself with a social category postdates the coming into the world of a subject whose experience is structured the way that it is, so unsurprisingly social identification is not only analytically independent of its pre-reflective ground, it is also temporally downstream of it.⁴

We know of no off-the-shelf sense of identity that has currency in contemporary philosophy that corresponds to this notion. As we have seen, it is not primarily or exclusively a social identity, at least not insofar as social identity is a matter of how one is perceived or identified by others. Being autistic may or may not be required for *numerical* identity, depending on one's theory of numerical personal identity. Would a person be multiplied or literally cease to exist if autism were cured by the snap of divine fingers? Maybe, maybe not. But even if so, recognizing that this is the case would be little to recognize what is distinctive about the hybrid sense of identity at work in autism.

In part, the difficulty here stems from a straightforward ambiguity in talk of 'identity.' On the one hand, identity simply refers to what one most essentially is. On the other hand, *identification* is something one *does* which may or may not rise to the level of partly constituting what one is. Sometimes what one identifies as becomes part of what one is, especially if it is central to one's self-understanding. But in autism, the relevant thing that one, in the first instance, *is*, is something that one need have no reflective awareness of at all in order for it to be the basic kind of pre-reflective ground that it is. The same cannot be said about *socially identifying as* autistic, yet doing so may or may not be central to one's self-understanding, depending on how centrally that facet of identity figures in one's overall self-understanding.

⁴ The importance of recognizing some basis for social and political identification in the ND movement has not gone unnoticed. Consider Chapman: "while it is important to recognize the positive significance of autism as a political identity, I am wary of reducing the nature of autism to a matter of group identification" (Chapman, 2020, p. 810).

社会和政治认同涉及一种认同性的行为，但经验的基本前反思性结构则未必如此。也就是说，与社会认同不同，经验的前反思性结构并不必然由任何将某种社会属性谓词化于自我的反思性判断行为所构成。

因此，我们必须仔细区分在孤独谱系中发现的经验的前反思结构，与基于经验如此结构化的社会或政治认同。前者是一种身份，因为它对一个人的本质至关重要；后者则是一种社会身份，是个人反思性地应用、并（部分）藉此确定其社会世界中位置的认同。与所有社会身份一样，归属感受到规范性调节，即成员被期望使其信念和行为符合群体特定规范。但由于作为孤独谱系的认同基础，在社会类别中具有非同寻常高度的第一人称属性，这些基础便具有了其独立的重要性。既然这些基础本身并非社会性的，我们就不得不承认，作为一种身份，孤独谱系包含这两个不同的要素：在孤独谱系中发现的经验的前反思结构，以及社会或政治认同；且两者都不能被简化为对方。此外，认同一个社会范畴所需的各种认知能力的获得，晚于一个经验结构已然如此的主体来到这个世界的时间，因此，社会认同不仅在分析上独立于其前反思基础，在时间顺序上也晚于它，这并不令人意外。⁴

据我们所知，当代哲学中并无现成的、通用的身份概念与此观念相对应。正如我们所看到的，它主要或并非完全是一种社会身份，至少在社会身份是关于一个人如何被他人感知或认定的意义上并非如此。成为孤独谱系人士是否是 `数值` 身份的必要条件，取决于个人的数值个人身份理论。如果孤独谱系被神之手一指而治愈，一个人是会增殖，还是字面意义上不复存在？也许，也许不会。但即便如此，认识到这种情况，也几乎无助于理解孤独谱系中运作的混合身份感的独特之处。

这里的困难部分源于谈论‘身份’时存在一个直接的模糊性。一方面，身份仅仅指的是一个人最本质的所是。另一方面，认同是一个人做的事情，它可能达到部分构成一个人所是程度，也可能达不到。有时，一个人所认同的会成为其自身的一部分，特别是当它对其自我理解至关重要时。但在孤独谱系中，一个人首先所是的那个相关事物，是一个人完全无需反思性意识就能成为的那种基本的前反思基础。对于社会性地认同为孤独谱系者却不能这样说，然而，这样做可能对其自我理解至关重要，也可能不，这取决于身份的这一面在其整体自我理解中占据的核心程度。

⁴ 认识到神经多样性运动中社会与政治认同的某种基础的重要性，并未被忽视。考虑一下 Chapman 的观点：“虽然认识到孤独谱系作为一种政治身份具有积极意义很重要，但我对将孤独谱系的本质简化为群体认同的问题持谨慎态度”（Chapman, 2020, p. 810）。

2.3 Autistic identity and HAP

The fact that autistic identity has these two aspects, i.e., a pre-reflective structuring of experience and a social or political identification, and that they are related in the way we have been sketching, makes it different from even other psychiatric identities. Consider, both by way of comparison, and to pre-empt an objection, the sort of identity Hanna Pickard argues is at work in addiction. According to Pickard (2021), addiction affects one's sense of self in a way that often renders treatments ineffective. Addiction can become a central aspect of how individuals perceive themselves, dominating their lives, thoughts, actions, social interactions, and relationships. Thus, she argues, the persistence of drug use among people who recognize its detrimental effects may not be primarily due to the irresistible nature of the drugs or a compulsion felt by the user, but rather may be partially due to a social identification with being an addict. In addition to any neurobiological effects caused by long-term drug use, drug-taking is reinforced by social reward, as it can be associated with companionship, camaraderie, or other forms of community among users. Pickard explains:

for people whose identity as an addict provides their main if not only source of social reward and community, structure and purpose in life, self-esteem, and sense of self and social identity, the act of imagining their life and identity in absence of their addiction is far from straightforward. (...) [W]ho would they be? Abandoning their identity as an addict represents an existential threat: there may be no alternative social identities or underlying stable, core sense of self to fall back on. (...) If it is not possible to imagine yourself without imagining being an addict, then you lack not only a rationale but also an imaginative basis for committing to a self-identification as a non-addict. If you make this commitment, it is as if you jump blind, with no certainty that you will survive the fall. This is terrifying, and renders utterly comprehensible why a person might choose to continue to be an addict rather than face this existential threat. (Pickard, 2021, p. 752).

Notice first that there is certainly some overlap between Pickard's sense of identity and the hybrid sense we have been explicating. Identification with a social kind which, in turn, structures, to a greater or lesser degree, one's practical outlook just is a major aspect of any social identity. It is precisely identity in this sense that, Pickard argues, could be a barrier to effective treatment. To overcome this problem and improve treatment efficacy, Pickard proposes adopting language in which the person's primary identity is distinguished from their social identity as a drug user, i.e., "to name drug use as a problem without making it an essential part of the person by grammatical design" (p.752).

In our terms, Pickard is arguing for the adoption of HAP for addiction, on the basis of pragmatic reasons. Could one similarly argue that HAP should be maintained for autism on similar grounds? We do not think so, and explaining why will further allow us to clarify the relevant sense of identity at work in our discussion.

First, Pickard's pragmatic proposal applies to entities or states which are such that cure and separation from the person is desirable. In the case of neurodivergences, at

2.3 孤独谱系身份与HAP

孤独谱系身份具有这两个方面，即经验的前反思结构和社会或政治认同，并且它们以我们一直勾勒的方式相关联，这一事实使其甚至不同于其他精神病学认同。为了比较并预先回应一个反对意见，请考虑Hanna Pickard认为在成瘾中起作用的那种身份。根据Pickard (2021)，成瘾以一种常使治疗无效的方式影响一个人的自我意识。成瘾可能成为个体如何看待自己的核心方面，主导其生活、思想、行动、社会互动和人际关系。因此，她认为，在认识到其有害影响的人群中，药物使用的持续可能主要不是由于药物的不可抗拒性或使用者的感受到的强迫，而可能是部分由于对成为成瘾者的社会认同。除了长期药物使用引起的任何神经生物学效应外，药物使用还通过社会奖励得到强化，因为它可能与使用者之间的陪伴、同志情谊或其他形式的社群相关联。Pickard解释道：

对于那些将成瘾者身份作为其主要（即便不是唯一）社会奖励和社群、生活结构与目标、自尊、以及自我感与社会身份来源的人来说，想象没有成瘾的生活与身份远非易事。(.....)[他/们]会是谁？放弃成瘾者身份意味着一种存在性威胁：可能没有替代性的社会身份或内在稳定、核心的自我感可以依靠。(.....) 如果无法想象自己不是成瘾者，那么你不仅缺乏理由，也缺乏想象基础来承诺自我认同为非成瘾者。若你做出这种承诺，就如同蒙眼跳跃，无法确定能否在坠落中幸存。这令人恐惧，也完全解释了为何一个人可能选择继续作为成瘾者，而非面对这种存在性威胁。(Pickard, 2021, 第752页)。

首先要注意的是，Pickard 所论述的身份与我们一直在阐释的混合意义身份之间，无疑存在某些重叠。认同一种社会类别，而该类别又在或多或少的程度上构建了个体的实践观，这本身就是任何社会身份的一个主要方面。Pickard 认为，正是这种意义上的身份认同，可能成为有效治疗的障碍。为了克服这个问题并提高治疗效果，Pickard 提议采用一种语言，将个体的主要身份与其作为药物使用者的社会身份区分开来，即“将药物使用命名为一个问题，而不通过语法设计使其成为个体本质的一部分”（第752页）。

用我们的术语来说，Pickard 是基于实用主义理由，主张对成瘾采用 HAP。那么，人们是否可以用类似的理由主张，对于孤独谱系也应基于类似理由维持 HAP？我们认为并非如此，而解释其原因将使我们进一步阐明我们讨论中所涉及的相关身份意义。

首先，Pickard的务实提议适用于那些治愈及与个体分离是可取之事的实体或状态。就神经殊异性而言，至少在此处讨论的那些，情况通常并非如此。

least those under discussion here, this is not generally the case.⁵ There is no legitimate reason to claim that one's actions, traits, thought processes, or habits are not part of one's identity in order to facilitate cure if cure is not desirable.

What underlies this response is the fact that, while there is overlap between the sort of identity Pickard has in mind and the one operative in our discussion so far, there are also important differences. As we have been emphasizing, the nature of autism as a structuring pre-reflective ground is the basis for the common social identification with the social kind 'autism'. Both aspects are relevant for the operation of autistic identity, but the former is more fundamental. It is unclear whether there is anything analogous in addiction. It would be very difficult to claim – and we do not understand Pickard to be claiming – that there is something about addiction which pervades all aspects of experience for those with addictions. Naturally, addiction may involve a certain *degree* of pre-conditioning of one's lived experience. For instance, the distinction between the relevant drug and other consumable things or activities takes on a particular salience in addiction. But because addiction is a disorder of action and volition, this degree of pre-conditioning is significantly more limited than what we find in autism. It only applies to the aspects of experience seen to be related to drug consumption, not to just any aspect of experience. This can no doubt constitute a wide associative array, but nevertheless will fall short of the pervasive structuring found in autistic experience.

Moreover, addiction is built up over time via a pattern of consumption, so it is an alteration to a non-addicted, pre-addicted person.⁶ Further, the pattern of consumption which leads to the acquisition of the addictive appetite⁷ is composed of discrete agential episodes (even if the pattern is not intentionally pursued *de dicto*), so it isn't plausible that it, or the appetitive state that it leads to, could be understood in non-agential terms. In contrast, voluntary agency is not part of the genealogy of autism and, as we have argued, autism is not an alteration to a pre-autistic, non-autistic person. Once again, the identification and elimination of addiction is desirable because the alterations to volition and agency that constitute its core are *undesirable*. Yet, the contention that neurodiverse conditions are not essentially undesirable is the main thrust of the ND movement.

So, for ND activists, being autistic is part of who one is in two senses: first, it is an ineliminable basic framework or ground of experience. Second, it is part of who one is in the same way that being a parent, being a Christian, or being a philosophy professor might be a part of who someone is. But the pre-social non-intentional former is the ground for the essentially social and reflexively tokened latter. These two aspects of autistic identity taken together are what we believe ND activists see as inconsistent with HAP.

⁵ We remain agnostic about whether it might be desirable for at least some people to no longer be neurodivergent.

⁶ Suppose we grant that a person can be born with an addiction, i.e., can be born with a cyclical appetite for a substance induced by exposure to that substance *in utero*. Even in such a case it is clear that such exposure is responsible for the change from of a fetus without addiction to a fetus with an addiction.

⁷ See e.g. Gibson (2024) for a philosophical discussion of addiction which embeds a review of the leading neuroscience underlying this feature of addiction.

⁵ 如果治愈并不可取，那么就没有正当理由声称一个人的行为、特质、思维过程或习惯不属于其身份的一部分，以便于进行治疗。

这一回应背后的实质是，尽管Pickard所设想的那种身份与我们迄今为止讨论中起作用的那种身份存在重叠，但也存在重要差异。正如我们一直强调的，孤独谱系作为一种构成性的前反思基础，其本质是与社会类别‘孤独谱系’产生共同社会认同的根基。这两个方面都与孤独谱系身份的运行相关，但前者更为根本。目前尚不清楚在成瘾中是否存在任何类似的东西。很难声称——我们也不认为Pickard在声称——成瘾中存在某种东西，能渗透到成瘾者体验的方方面面。当然，成瘾可能涉及对个人生活体验的某种程度的前置条件化。例如，成瘾中，相关药物与其他可消费物品或活动之间的区别会变得格外突出。但由于成瘾是一种行为和意志障碍，这种前置条件化的程度远比我们在孤独谱系中发现的情况更为有限。它只适用于被认为与药物消费相关的体验方面，而非体验的任何方面。这无疑可以构成一个广泛的联想阵列，但仍将不及孤独谱系体验中那种无处不在的结构化程度。

此外，成瘾是通过一种消费模式随时间建立起来的，因此它是针对一个未成瘾的、成瘾前的人的改变。⁶ 再者，导致获得成瘾性欲望的消费模式⁷ 是由离散的主体性事件构成的（即使该模式并非有意地追求*de dicto*），因此，认为它或它所导致的欲望状态可以用非主体性的术语来理解，是站不住脚的。相比之下，自愿的主体性并非孤独谱系的起源，而且正如我们所论证的，孤独谱系并非对一个孤独谱系前的、非孤独谱系的人的改变。再次强调，识别和消除成瘾是可取的，因为构成其核心的对意志和主体性的改变是不可取的。然而，神经多元状况本质上并非不可取的观点，正是神经多样性运动的主要推力。

因此，对于神经多样性活动家而言，拥有孤独谱系身份是一个人之所以为人的一部分，这体现在两个方面：首先，它是一个不可消除的基本经验框架或基础。其次，它是个人身份的一部分，就如同为人父母、身为基督徒或担任哲学教授可能构成某人身份的一部分一样。但先于社会存在的、非意向性的前者，正是本质上具有社会性且可被反思性标记的后者的基础。我们认为，神经多样性活动家正是将孤独谱系身份的这两个方面结合起来，视为与HAP不相容的。

⁵ 我们对于是否至少对某些人而言，不再具有神经多元特质可能是可取的，仍持不可知态度。⁶ 假设我们承认一个人可能天生就有成瘾，即，可能天生就具有一种对某种物质的周期性欲望，这种欲望是由在子宫内暴露于该物质所诱发的。*in utero*。即使在这种情况下，显然这种暴露导致了胎儿从无成瘾状态到有成瘾状态的改变。⁷ 例如，参见 Gibson (2024) 中关于成瘾的哲学讨论，其中包含了对构成成瘾这一特征的神经科学主流研究的综述。

So, HAP is not defensible for autism on the grounds that Pickard invokes for its defense in addiction, but we may still wonder whether some ND activists might want to maintain HAP on other grounds. After all, the movement is far from homogeneous. Indeed, there are two major forms of heterogeneity within the movement which are relevant in this connection, but both are ultimately compatible with the fundamental commitment to rejecting HAP.

First, while rejecting a biomedical model of categories of neurodivergence—which model views them as biological abnormalities treatable with pharmacological interventions—is quite widespread within the movement, there is some disagreement on what alternative framework should be used to understand neurodivergences. Second, the extent to which an individual identifies with a given psychiatric label is a matter of degree. Let us consider these related complexities.

Most ND activists have pushed for understanding categories of neurodivergence using the social model of disability according to which people are not disabled by psychiatric conditions, but by social barriers (Oliver, 1983, 2013). But among them, some commit to a “strong” social model of disability, which attributes the harm suffered by autistic people to a non-inclusive and predominantly neuronormative environment (Catala et al., 2021; Legault et al., 2021), whereas others commit to a weaker version (Dwyer, 2022). For those adopting a strong social model, the rejection of HAP is obvious. They argue for the importance of considering autism as part of one’s identity, which makes one vulnerable to social norms that prevent full inclusion and acceptance. But some who adopt a weaker social model claim that the strong reading can only make sense for autistic people who do not have intellectual or other impairments and cannot apply to those for whom autism can be inherently harmful (Angulo-Jiménez & DeThorne, 2019). For these theorists, there could be a pragmatic concern about rejecting HAP: the lack of a clear separation between a person and their psychiatric label might hinder the medical system’s ability to effectively provide care, potentially leaving those in need without resources. Would the proponents of a weaker social model accept HAP? We think they would not. Those who claim that autistic traits inherently prevent someone from being included in society may challenge the positive value of social identification. But this does not mean they reject the idea of identification altogether. In fact, those who adopt a weaker social model often identify particularly strongly with their autism, often insisting that some of the difficulties associated with autism are biologically grounded (Angulo-Jiménez & DeThorne, 2019). Regardless of whether such claims are ultimately defensible, it is clear that such activists view their autism as ineliminable and basic. So even if accepting HAP might seem pragmatically useful to those who reject a strong social model (in much the same way Pickard supports HAP in addiction for therapeutic purposes), autism’s role in the pre-reflective structuring of experience makes doing so infeasible.

There is a wide range of perspectives within the ND movement. Some explicitly promote Neurodiversity as a social identity and embrace what they see as its a unique culture (Sinclair, 2005; Beck, 2023); others emphasize how neurodivergence interacts with other social categories, influencing experiences and advocacy goals (Baga-tell, 2007; Brown, 2017); still others highlight how societal structures often make life difficult for autistic people, even if they embrace their identity (Milton, 2012). This

因此，HAP无法像Pickard为成瘾辩护时所援引的理由那样，在孤独谱系问题上站得住脚。但我们仍可能疑惑，是否有些神经多样性活动家会基于其他理由而坚持HAP。毕竟，这场运动远非铁板一块。事实上，运动内部存在两种主要的异质性形式与此相关，但两者最终都与拒绝HAP这一根本承诺相容。

首先，虽然拒绝将神经特殊性类别视为可用药物干预治疗的生物异常的生物医学模型在该运动中相当普遍，但对于应使用何种替代框架来理解神经特殊性，存在一些分歧。其次，个体对特定精神诊断标签的认同程度是一个程度问题。让我们来思考这些相关的复杂性。

大多数神经多样性活动家都推动使用残障社会模式来理解神经特殊性类别，根据该模式，人们并非因精神状况而致残，而是因社会障碍所致 (Oliver, 1983, 2013)。但在他们之中，有些人信奉一种“强”残障社会模式，该模式将孤独谱系群体所遭受的伤害归因于非包容性且主要由神经规范性主导的环境 (Catala等, 2021; Legault等, 2021)，而另一些人则信奉一种弱版本 (Dwyer, 2022)。对于那些采用强社会模式的人来说，拒绝HAP是显而易见的。他们主张将孤独谱系视为个人身份的一部分的重要性，这使人容易受到阻碍完全包容和接纳的社会规范的影响。但一些采用弱社会模式的人声称，强解读只对那些没有智力或其他障碍的孤独谱系群体有意义，无法适用于那些孤独谱系可能本身就有的人 (Angulo-Jiménez与DeThorne, 2019)。对这些理论家而言，拒绝HAP可能存在一种实用性的担忧：个人与其精神诊断标签之间缺乏明确区分，可能会阻碍医疗体系有效提供护理的能力，可能使有需要者无法获得资源。弱社会模式的支持者会接受HAP吗？我们认为他们不会。那些声称孤独特质本身会阻碍个人融入社会的人，可能会挑战社会认同的积极价值。但这并不意味着他们完全拒绝认同的观念。事实上，那些采用弱社会模式的人通常特别强烈地认同其孤独谱系身份，常常坚持认为与孤独谱系相关的某些困难具有生物学基础 (Angulo-Jiménez与DeThorne, 2019)。无论这些主张最终是否站得住脚，很明显，这些活动家将他们的孤独谱系视为不可消除且根本性的。因此，即使接受HAP对于那些拒绝强社会模式的人来说可能看起来具有实用性益处（这与Pickard在成瘾领域为治疗目的而支持HAP的方式非常相似），但孤独谱系在经验的前反思结构中的作用使得这样做并不可行。

神经多样性运动内部存在广泛的观点光谱。一些人明确将神经多样性作为一种社会身份加以推广，并拥抱他们所认为的独特文化 (Sinclair, 2005; Beck, 2023)；另一些人则强调神经特殊性如何与其他社会类别相互作用，影响体验和倡导目标 (Baga-tell, 2007; Brown, 2017)；还有一些人着重指出，即使孤独谱系群体接纳自己的身份，社会结构也常常使他们的生活变得困难 (Milton, 2012)。这种

diversity of perspectives is well recognized and acknowledged within the ND movement (Kapp, 2023). A low degree of connection with an autistic identity can weaken social and political identification. However, none of this should be construed as a reason to embrace HAP; as long as one shares in the basic nature of autistic experience, hypostatic abstraction is contraindicated.

2.4 Neurodiversity and anti-psychiatry

In the remainder of the paper, we argue that this understanding of the central demand of the ND movement has a number of consequences. Notably, it shows that important criticisms rest on misunderstandings, which we come to in the next section. Another consequence we would like to highlight presently. Understanding both Mad activism and the ND movement as rejecting HAP allows us to distinguish these movements from the anti-psychiatry movement, but also to see some key connections between them. Anti-psychiatry is perhaps best understood at the outgrowth of a kind of critical theory, summoning skepticism of the psychiatric establishment via a hermeneutics of suspicion. Although it would be very easy to think that the ND movement, because of its sometimes-antagonistic posture towards the psychiatric establishment, is fundamentally an outgrowth of this same impulse, we think that is a mistake, and our analysis show why this is the case. ND activists are acting in defense of their identities and often frame the cause in terms of justice, but they are not fundamentally committed to the claim that psychiatry, as such, is, for instance, a mere instrument of social control (Chapman, 2024).

For ND activists, the perceived need to act in defense of autistic identity does not come from a Foucaultian skepticism about psychiatric normativity or a Szaszian insistence that only verifiably organic pathology is within the ambit of medicine. (Indeed, as we have seen, some ND activists are perfectly happy to accept that autism has a neurobiological grounding, even if we don't know precisely what it is.) Rather, it rests on a worry about the importation of a notion like hypostatic abstraction from mathematics to a hybrid human-natural science such as psychiatry. This worry is notably expressed by Monique Botha, an autistic researcher in psychology, who argues research has a tendency to dehumanize and objectify autistic people: "If you continually describe autistic people through subhuman terms, then you will allow for subhuman treatment" (Botha, quoted in Rodríguez, 2023). This suspicion, we think, is a large part of what is animating the ND movement. Activists are primarily acting self-defensively. This involves a certain degree of skepticism about psychiatric normativity, but it needn't be nearly as general or as diffuse as one finds in Szasz, for instance. Indeed, the sweeping arguments given by Szasz for the non-existence of mental illness can very easily be seen to be in tension with the goals of the ND movement, especially as we understand them. Two of the main arguments given by Szasz for the non-existence of mental disorders rely respectively on the claims that (1) mental disorders are not forms of organic pathology and (2) mental disorders, having functional rather than structural criteria, are such that it is impossible to distinguish genuine from ersatz instances of them. (Szasz 1982, p.13, Ch. 1) Both of these claims, and the arguments built upon them, can be easily rejected by the ND movement.

视角的多样性在神经多样性运动内部已得到广泛认可和承认 (Kapp, 2023)。与孤独谱系身份的低度联结可能削弱社会与政治认同。然而, 这不应被理解为拥抱HAP的理由; 只要个体共享孤独谱系体验的基本性质, 实体抽象就是不适用的。

2.4 神经多样性与反精神病学

在本文的剩余部分, 我们认为, 对神经多样性运动核心诉求的这种理解会带来若干后果。值得注意的是, 这表明一些重要的批评是基于误解, 我们将在下一节讨论这一点。我们目前想强调的另一个后果是: 将疯狂行动主义和神经多样性运动都理解为对 HAP 的拒斥, 使我们能够区分这些运动与反精神病学运动, 同时也看到它们之间的一些关键联系。反精神病学或许最好被理解为一种批判理论的产物, 它通过怀疑诠释学唤起对精神病学体系的怀疑。尽管很容易认为, 神经多样性运动因其有时对精神病学体系的对抗姿态, 从根本上说也是同一冲动的产物, 但我们认为这是一个错误, 我们的分析表明了原因。神经多样性活动家是在捍卫他们的身份, 并常常将这项事业置于正义的框架下, 但他们从根本上并不认同“精神病学本身仅仅是社会控制的工具”这类主张 (Chapman, 2024)。

对于神经多样性活动家而言, 捍卫孤独谱系身份的行动需求, 并非源于福柯式怀疑对精神病学规范性的质疑, 也非萨斯式坚持认为只有可验证的器质性病理才属于医学范畴。(事实上, 正如我们所见, 一些神经多样性活动家完全乐于接受孤独谱系具有神经生物学基础, 即便我们尚不确切知晓其具体机制。)相反, 这种需求源于一种担忧: 担心像实体抽象这样的概念会从数学领域被引入精神病学这类混合了人文与自然的科学中。这种担忧在心理学领域的孤独谱系研究者Monique Botha那里得到了显著表达, 她认为研究倾向于非人化和物化孤独谱系群体: “如果你持续用非人的术语描述孤独谱系群体, 那么你就会允许非人的对待方式” (Botha, 转引自Rodríguez, 2023)。我们认为, 这种疑虑在很大程度上是驱动神经多样性运动的力量。活动家们主要是出于自卫而行动。这包含了对精神病学规范性的一定程度的怀疑, 但这种怀疑无需像在萨斯那里那样普遍或分散, 例如。事实上, 萨斯提出的关于精神疾病不存在的全面论点, 很容易被视为与神经多样性运动的目标相冲突, 尤其是我们理解中的那些目标。萨斯否定精神障碍存在的两个主要论点分别基于以下主张: (1) 精神障碍并非器质性病理的形式; (2) 精神障碍具有功能性而非结构性标准, 因此不可能区分其真实实例与仿造实例。 (Szasz 1982, p.13, Ch. 1) 神经多样性运动可以轻易拒绝这两项主张以及基于它们构建的论点。

We now want to show that much criticism of the ND movement implicitly relies on HAP or fails to acknowledge that it is being rejected. This involves perpetuating a fundamental misunderstanding of the ND movement's core objectives. In the remainder of the paper, we consider some criticisms brought against the ND movement (Sect. 3.1), then respond to these criticisms (Sect. 3.2.), and explain what we think really motivates them (Sect. 4). This will allow us to motivate a reconciliation between the ND movement and the psychiatric establishment.

3 How to understand the critics

3.1 Four influential criticisms

Criticisms of ND abound. Who are the critics and what are they worried about? For clarity of exposition, we separate the worries into four categories: criticisms focusing on (1) destigmatization and the social model of disability, (2) de-pathologizing, (3) inclusivity, and (4) social justice.

(1) Some researchers in psychology and neuroscience reject the reliance on alternative models of neurodiverse conditions such as the social model of disability by proposing that such a move is not necessary to achieve one of the ND movement's key aims, viz., destigmatization. In principle, this claim could be made from any number of different perspectives; perhaps it is just true that destigmatization does not require the radical reconceptualization asked for by ND activists, even if it might require some alteration to the theoretical status quo. But, as we find this line of criticism in the literature, it is made in defense of the biomedical model (Casanova & Widman, 2021; Loo et al., 2021), and thus becomes the claim that the aims of the ND movement (or at least the aim of destigmatization) can be achieved from within the biomedical model. According to such a view, people can be said to have disorders that harm them without being victims of stigmatization. The idea here seems to be that the ND movement is asking for too much in demanding to de-pathologize psychiatric categories when society could simply work at becoming more inclusive. ND activists would therefore be wrong to ask for a reconceptualization of psychiatric categories into a new framework such as the social model of disability.

(2) Other critical responses come from philosophers and scientists who think that seeing autism as a pathology should play *in favor* of advocating for the interests of autistic people, rather than *against* their inclusion, thereby trying to redirect some of the demands of ND activists into considering autism a pathology (Shields & Beversdorf, 2021).

(3) There are also parents, educators, social workers, and other allies whose worries are reflected in the public discourse. These groups demand clarity on how the ND movement can accommodate the reality of people who need therapeutic help and who worry that de-pathologizing will lead to a loss of access to therapeutic resources. Some neurodivergent people themselves claim that the demands are not representative of their own needs. For instance, Kansen (2017) says: "many of us aren't high-functioning enough to benefit from depathologizing autism. The neurodiversity movement doesn't have much to say about lower-functioning autistics." These

我们现在想要表明，对神经多样性运动的许多批评，要么隐晦地依赖于HAP，要么未能承认该运动已将其摒弃。这导致了对神经多样性运动核心目标的一种根本性误解持续存在。在本文余下部分，我们将考察针对神经多样性运动的一些批评（第3.1节），然后回应这些批评（第3.2节），并解释我们认为真正驱动这些批评的动机是什么（第4节）。这将使我们能够推动神经多样性运动与精神病学体系之间的和解。

3 如何理解批评者

3.1 四种有影响力的批评

针对神经多元（ND）的批评比比皆是。批评者是谁？他们担忧什么？为阐述清晰，我们将这些担忧分为四类：聚焦于（1）去污名化与残障社会模式，（2）去病理化，（3）包容性，以及（4）社会正义的批评。

（1）一些心理学和神经科学领域的研究者反对依赖神经多元状况的替代模型（例如残障社会模式），其理由是proposing这种转变并非必要y以实现神经多样性运动的一个关键目标，即去污名化。原则上，这一主张可以从任何数量的不同视角提出；或许仅仅是去污名化并不需要神经多样性活动家所要求的彻底概念重构，即使它可能需要对理论现状进行一些调整。但是，正如我们在文献中发现这种批评思路，它是为生物医学模型辩护而提出的(Casanova & Widman, 2021; Loo 等人, 2021)，因此就变成了这样一种主张：神经多样性运动的目标（或至少是去污名化的目标）可以在生物医学模型内部实现。根据这种观点，人们可以被认为患有对其有害的障碍，而无需成为污名化的受害者。这里的想法似乎是

有人认为，神经多样性运动要求将精神病学分类去病理化是要求过高，而社会本可以努力变得更包容。因此，神经多样性活动家若要求将精神病学分类重新概念化为残障社会模式等新框架，便是错误的。

（2）其他批判性回应来自哲学家和科学家，他们认为将孤独谱系视为病理学应当有利于倡导孤独谱系群体的利益，而非阻碍他们的融入，从而试图将神经多样性活动家的部分诉求转向将孤独谱系视为一种病理(Shields & Beversdorf, 2021)。

（3）还有一些家长、教育工作者、社会工作者及其他盟友，他们的担忧体现在公众讨论中。这些group方要求明确y关于如何神经多样性运动可以容纳那些需要治疗帮助、并担心去病理化会导致失去治疗资源获取渠道的人们的现实。一些神经殊异人群自己声称，这些要求并不能代表他们自身的需求。例如，Kansen (2017) 说道：“我们中的许多人并非功能足够高，能够从孤独谱系去病理化中受益。神经多样性运动对于功能较低的孤独谱系者鲜有论述。

criticisms vary in their degree of resistance to the ND movement, but most consider its demands non-applicable to people who cannot use verbal language or who have other significant disabilities (Russell, 2020), sometimes considered “low functioning”, in accordance with the DSM conceptualization of different levels of “severity” of autism⁸. The inclusivity demanded by the ND movement, so the worry goes, does very little to address the needs of some in the neurodivergent community and could, on the contrary, imperil their access to what therapeutic supports they currently have.

(4) Some criticisms are coming from within psychiatry itself, where some have felt the need to defend psychiatrists from the alleged injustices that psychiatry has been accused of committing against service users. Fricker, who coined the term “epistemic injustice”, defines it as a wrong done to someone in their capacity as a knower (2007). The concept has recently been intensively applied in the context of psychiatry to alleged prejudices against people in psychiatric settings (Pantazakos & Arnaud, 2024). This, in turn, has led some to defend the practice against these allegations. For instance, Kious et al. (2023, p.1) claim that, in general, “The concept of epistemic injustice does not add significantly to existing standards of good clinical practice, and (...) it could produce changes in practice that would be deleterious. Psychiatrists should resist calls for changes to clinical practice based on this type of criticism.” In response to claims that psychiatry perpetrates epistemic injustice, Kious et al. claim that making changes to these practices could end up harming those it serves by “sidelin[ing] principles of good clinical reasoning, and substituting them with a pursuit of social justice that diminishes the value of skepticism and critical thinking” (Ibid., p. 4). Although this claim does not directly address the ND movement, it can easily be understood in such a way that neutralizes the demands of the ND movement by claiming that when it comes to a conflict between psychiatry and social justice, the aims of psychiatry should take precedence.

We think these worries and criticisms fail to recognize that the central demand of the ND movement is the rejection of HAP. If we are right, much of this apparent disagreement is founded on a misapprehension. While one can find explicit and direct opposition to the distinctively *political* aspects of the ND movement (such as by those who stoke fears about an “autism epidemic”), we believe that this kind of genuine political disagreement is rarer than one might think from examining the debate.⁹ This ought to strike us as good news. If the ground can be cleared, the debate can proceed more productively.

⁸ Note however that the idea of severity scales has been controversial precisely because it threatens the necessary unity for self-advocacy. In this connection see Kapp (2023).

⁹ Ultimately, all or most aspects of the ND movement, understood as we propose, are political, even those that seem to concern only the methods of psychiatric practice. For instance, activist-led objections to Applied Behavior Analysis (ABA) therapy are not solely about the method itself, but about the intention to cure autism. While practitioners might still want to defend ABA for its effectiveness in certain contexts, that commitment in itself does not preclude alignment with the political aims of the ND movement. Of course, it does not guarantee alignment with them either; genuinely political disagreement may persist, but all parties benefit from being clear about its nature.

这些批评对神经多样性运动的抵制程度各不相同，但大多认为其诉求不适用于那些无法使用言语或具有其他严重障碍的人士 (Russell, 2020)，这些人有时被称为“低功能”，依据的是DSM对孤独谱系不同“严重程度”等级的概念化。⁸ 担忧在于，神经多样性运动所要求的包容性，对满足神经殊异社群中部分人士的需求作用甚微，反而可能危及他们目前所能获得的治疗支持。

(4) 一些批评来自精神病学内部，其中一些人认为有必要为精神科医生辩护，驳斥精神病学被指控对服务用户所犯下的所谓不公。Fricker创造了“认知不正义”这一术语，将其定义为对一个人作为认知者的能力所造成的伤害 (2007)。最近，这一概念被广泛应用于精神病学领域，针对精神病学环境中对人们的所谓偏见 (Pantazakos & Arnaud, 2024)。这反过来又促使一些人为此类实践辩护，以应对这些指控。例如，Kious等 (2023年, 第1页) 声称，总体而言，“认知不正义的概念并未对现有良好临床实践标准做出显著补充，并且 (……) 它可能引发对实践的有害改变。精神科医生应抵制基于此类批评而改变临床实践的呼声。”针对精神病学犯有认知不正义的指控，Kious等声称，改变这些实践最终可能通过“边缘[化]良好临床推理原则，并以追求社会正义来替代它们，从而削弱怀疑精神和批判性思维的价值” (同上, 第4页)，从而伤害其所服务的人群。尽管这一主张并未直接针对神经多样性运动，但很容易被理解作为一种旨在消解神经多样性运动诉求的方式，即声称当精神病学与社会正义发生冲突时，精神病学的目标应优先考虑。

我们认为，这些担忧和批评未能认识到，神经多样性运动的核心诉求是拒绝HAP。如果我们的观点正确，那么这种表面上的分歧很大程度上是基于一种误解。虽然人们可以找到对神经多样性运动中独特的政治方面的明确而直接的反对 (例如那些煽动对“孤独症流行”恐惧的人)，但我们相信，这种真正的政治分歧比审视辩论时可能认为的要罕见得多。⁹ 这应当让我们感到振奋。如果能扫清障碍，辩论就能更富有成效地进行下去。

⁸ 然而需要注意的是，严重程度量表的构想之所以备受争议，恰恰是因为它威胁到了自我倡导所必需的团结。关于这一点，可参见Kapp (2023)。⁹ 最终，如我们所阐释的，神经多样性运动的所有或大部分方面都是政治性的，即便是那些看似仅涉及精神病学实践方法的部分。例如，由活动家主导的对应用行为分析 (ABA) 疗法的反对，并不仅关乎方法本身，更关乎其治愈孤独谱系的意图。尽管从业者可能仍想为ABA在某些情境下的有效性辩护，但这种承诺本身并不妨碍其与神经多样性运动的政治目标保持一致。当然，这也不保证必然一致；真正的政治分歧可能持续存在，但各方都能从厘清其本质中受益。

3.2 What the critics get wrong

On one way of reading those who make criticism (1), they are making one basic mistake: the attribution of the social model of disability to the ND movement as a whole. As we have already seen, some ND activists reject such a model, and among the rest, there is considerable variation on what version to adopt.

Perhaps a more charitable reading of (1) is primarily as an objection to rejecting the biomedical model, rather than as an objection to the adoption of any positive alternative. On this reading, the critics are motivated to distinguish between the biomedical model, and the sorts of stigmatization, exclusion, and other negative consequences that *happen* to come along with the use of the biomedical model, in our society as it currently is. The idea is that the biomedical model, when applied to neurodivergence, does not *necessarily* lead to the negative social consequences that ultimately animate activists, including ND activists. Rejecting the biomedical model would thus be unnecessary or unjustified.

According to this reading, society and psychiatry should work at destigmatizing and including people with a psychiatric label without having to claim that these psychiatric terms constitute identities. This is unobjectionable as far as it goes. But it overlooks the agenda of the ND movement, which is not merely about destigmatization and inclusion within existing frameworks but about fundamentally questioning what is considered a disorder in the first place.¹⁰ Indeed, this reading of criticism (1) simply involves *assuming* HAP and failing to recognize that the rejection of HAP is inherent to the demands of ND activism: why would it be important to give the biomedical model of neurodivergences a kinder, gentler face if not ultimately for the purposes of rehabilitating the idea that neurodivergences ought to be eliminated by cure? Since the ND movement's rejection of HAP is rooted in the belief that neurodivergences are not mere deviations from a social norm but rather natural variations within the human condition (Jaarsma & Welin, 2012), the movement seeks to shift the discourse from one of disorder and deviation to one of (as we have seen, optionally) disability, diversity and variation, i.e., to a discourse compatible with the rejection of HAP. By *assuming* HAP, the criticism fails to engage with the ND movement's idea that adopting a biomedical model of neurodivergence contributes to, not just the stigmatization of neurodivergent people, but also to reinforcing problematic frameworks that treat neurodivergences as separable entities rather than integral components of people's identities.

Those who make criticism (2) about de-pathologizing argue that to the extent that the ND movement advocates for the de-pathologization of neurodiversity, it risks at the same time undermining claims to treatment, resources, care, and attention, or (as Shields and Beversdorf (2021) emphasize) legal recognition. As Shields and Beversdorf say, "Our argument thus raises a dilemma for the neurodiversity movement: advocating for the rights and interests of individuals with autism may require viewing autism as a condition that can be inherently disabling (at least for some individuals)" (p. 126).

¹⁰ See also Gibson and Arnaud (Forthcoming), for discussion of the limits of destigmatization alone as a goal for the ND movement.

3.2 批评者错在何处

按照一种解读批评(1)的方式,他们犯了一个基本错误:将残障社会模式归因于整个神经多样性运动。正如我们已经看到的,一些神经多样性活动家拒绝这种模式,而在其余的人中,对于采用何种版本也存在相当大的差异。

或许对观点(1)一种更善意的解读是:它主要反对的是摒弃生物医学模型,而非反对采纳任何积极的替代方案。按此解读,批评者的动机在于区分生物医学模型,与那些在我们的现实社会中恰好伴随生物医学模型使用而产生的污名化、排斥及其他负面后果。其观点是,当生物医学模型应用于神经特殊性时,并不必然导致那些最终驱动活动家(包括神经多样性活动家)的负面社会后果。因此,摒弃生物医学模型将是多余或不合理的。

根据这种解读,社会与精神病学应致力于去污名化并接纳拥有精神诊断标签的人群,而无需声称这些精神病学术语构成了身份认同。就其本身而言,这无可厚非。但它忽视了神经多样性运动的议程,该运动不仅关乎在现有框架内去污名化和包容,更关乎从根本上质疑究竟什么才应被视为障碍。¹⁰事实上,对批评(1)的这种解读仅仅涉及预设了HAP,且未能认识到,摒弃HAP是神经多样性运动行动主义诉求的固有之义:如果不是为了最终恢复"神经特殊性应通过治愈来消除"这一观念,那么赋予神经特殊性的生物医学模型一个更温和、更友善的面孔又有何重要?由于神经多样性运动对HAP的摒弃植根于这样一种信念,即神经特殊性并非仅仅是偏离社会规范,而是人类境况中自然的变异(Jaarsma与Welin, 2012),该运动寻求将话语从障碍与偏离转向(正如我们所见,可选择性的)残障、多样性与变异,即转向一种与摒弃HAP相容的话语。通过预设HAP,该批评未能触及神经多样性运动的核心观点:采纳神经特殊性的生物医学模型,不仅助长了神经特殊人群的污名化,还强化了将神经特殊性视为可分离的实体而非人身份认同中不可或缺组成部分的有问题框架。

对去病理化提出批评(2)的人认为,神经多样性运动在倡导神经多样性去病理化的同时,也可能削弱了对治疗、资源、关怀、关注的主张,或者(如Shields and Beversdorf (2021)所强调的)法律上的承认。正如Shields and Beversdorf所说:"因此,我们的论点给神经多样性运动提出了一个两难困境:倡导孤独谱系个体的权利和利益,可能需要将孤独谱系视为一种本质上可能致残的状况(至少对某些个体而言)"(第126页)。

¹⁰ 另请参见Gibson和Arnaud(即将出版),他们讨论了仅将去污名化作为神经多样性运动目标的局限性。

First, this criticism seems to involve rather explicit adherence to HAP. The ND movement, especially as we are construing its main thrust, is not homogeneously committed to denying that neurodivergences are disabling. Whether they are *inherently* disabling also does not seem to be germane when the claims to treatment, resources, etc. are made relative to a set of social institutions and practices (those very institutions and practices against which the claims are made.) We can see this more clearly when we interpret ND theorists as denying HAP. The critics in this category seem to be imagining that any demand for de-pathologization would bring with it the claim that the conditions in question are not disadvantageous (relative to a context.) But interpreted in the way we are suggesting, there is no reason to think this implication would go through. Insofar as we are right, ND activists are asking for a reconceptualization of the relation between a person and their diagnosis. This does not mean that the resulting identity category is not disadvantageous to occupy. In fact, de-pathologizing, viewing something as partly constitutive of a certain kind of identity, and perhaps labeling it as a disability instead of a disorder, are all compatible with acknowledging inherent harm. Hughes proposes that even a strong identification with a psychiatric category—like Sinclair’s claim that autistic people would no longer “exist” without autism (1993)—is compatible with the idea that autism is inherently detrimental (Hughes, 2021). His idea is that identifying with autism does not preclude it from being harmful. This could be because one might be mistaken about how the absence of autism would affect their well-being, or it could simply be that one is practically committed to something which has well-being costs, much like how certain career choices or the adoption of a strictly ascetic religious practice might also involve sacrifices to one’s well-being. Note that in these cases too, the extent to which a person identifies with these choices or practices can vary significantly. A strong identification with a particular role or belief does not necessarily equate to attributing a high value to it. One might primarily identify as an academic and yet perceive this career choice as inherently detrimental. Similarly, even when religious beliefs are deeply ingrained and not perceived as mere choices, one can consider them a central part of their identity while still viewing them as harmful.

Similarly, in response to (3), the criticism about inclusivity, we claim that those who worry that capitulating to the demands of ND activists would exclude those who need therapeutic care make a false presupposition. They impute to the ND movement the claim that neurodivergences should no longer be *among the objects of psychiatry*, i.e., that autism, ADHD and other categories are not legitimate psychiatric categories and should not ground access to treatment or other clinical encounters. But, if we are correct, this is not what is being demanded. De-pathologizing does not entail de-psychiatrizing. What might well be required is a reconciliation between the practices of psychiatry and the central demand of ND activists, but as we have already seen, it is a mistake to interpret the ND movement in anti-psychiatry terms, and such a reconciliation is desirable in any case. None of the arguments considered here show, or even attempt to show, that such a reconciliation is neither possible nor desirable. In fact, as some ND activists have claimed, while it is not the role of psychiatry to spearhead fundamental social changes, there is room for clinical encounters that are “neurodivergence-informed” (Chapman and Botha, 2022). ND-informed approaches have considered the heterogeneity of these categories and promote inclusion through

首先, 这种批评似乎相当明确地遵循了HAP。神经多样性运动, 尤其是按照我们对其主要主旨的理解, 并非一致地否认神经殊异性具有致残性。它们是否本质上具有致残性, 在主张治疗、资源等是相对于一套社会制度和实践 (正是那些主张所针对的制度和实践) 而提出时, 似乎也无关紧要。当我们把神经多样性理论家解释为否定HAP时, 可以更清楚地看到这一点。这一类的批评者似乎认为, 任何去病理化的要求都会伴随着声称所讨论的状况 (在特定背景下) 并非不利。但按照我们建议的方式来解释, 没有理由认为这种含义会成立。只要我们的理解是正确的, 神经多样性活动家就是在要求重新概念化一个人与其诊断之间的关系。这并不意味着由此产生的身份类别不具有不利之处。事实上, 去病理化、将某事物视为某种身份的部分构成要素, 或许将其标记为残障而非障碍, 所有这些都与承认内在伤害是兼容的。Hughes提出, 即使是对某个精神病学类别的强烈认同——比如Sinclair声称没有孤独谱系, 孤独谱系群体将不复“存在” (1993) ——也与孤独谱系本质上具有损害性的观点兼容 (Hughes, 2021)。他的观点是, 认同孤独谱系身份并不排除其有害的可能性。这可能是因为一个人可能错误地判断了没有孤独谱系会如何影响其福祉, 或者也可能只是因为一个人实际投身于某种需要付出福祉代价的事物, 就像某些职业选择或采用严格的苦行宗教实践可能也会涉及对个人福祉的牺牲一样。请注意, 在这些情况下, 个人对这些选择或实践的认同程度也可能差异很大。对特定角色或信仰的强烈认同, 并不一定等同于赋予其很高的价值。一个人可能主要认同学者身份, 却仍认为这一职业选择本质上是有害的。同样, 即使宗教信仰根深蒂固且不被视为单纯的选择, 一个人也可以将其视为自我身份的核心部分, 同时仍认为它们是有利的。

同样, 针对 (3) 关于包容性的批评, 我们声称, 那些担心屈服于神经多样性活动家的要求会排除需要治疗护理的人的观点, 是基于一个错误的预设。他们归咎于神经多样性运动的主张是: 神经殊异性不应再属于精神病学的对象, 即孤独谱系、注意缺陷多动障碍和其他分类并非合法的精神病学分类, 不应作为获得治疗或其他临床接触的基础。但是, 如果我们对的, 这并非其要求。去病理化并不意味着去精神病学化。可能真正需要的是精神病学的实践与神经多样性活动家的核心诉求之间的调和。然而, 正如我们已经看到的, 将神经多样性运动解读为反精神病学是一种误解, 而这种调和无论如何都是可取的。这里所讨论的论点均未表明, 甚至未曾试图表明, 这种调和既不可能也不可取。事实上, 正如一些神经多样性活动家所言, 虽然推动根本性的社会变革并非精神病学的职责, 但存在采用“神经殊异知情” (Chapman 和 Botha, 2022) 方式进行临床接触的空间。神经多样性知情方法已考虑到这些分类的异质性, 并通过

the recognition of a diversity of needs. This can be true regardless of whether one acknowledges a distinction between levels of ‘functioning’ in autism. Nothing prevents medical assistance from being useful for those who see their diagnosis as being partly identity-constitutive.

As we mentioned above, those who advance (4) and are motivated to defend psychiatry against criticisms based in social injustice need not be directly concerned with the ND movement. Nevertheless, we find here the idea that it is part of the nature of some psychiatric conditions, as diagnosed, that the epistemic credentials of their bearers are diminished. As such, a general worry that psychiatry perpetrates epistemic injustice is unwarranted because, the thought goes, it is not *injustice* to downgrade someone’s epistemic credentials when that person’s epistemic capacities are themselves diminished. While Kious et al. do not explicitly commit themselves to HAP for neurodivergences, what they do say puts them in a position where rejecting HAP would be problematic. They suggest that certain psychiatric categories imply downgraded epistemic credentials for those to whom the category applies. If we combine that with the rejection of HAP for the relevant categories, we arrive at the problematic conclusion that some aspects of one’s practical or social identity warrant downgraded epistemic credentials for those with that identity. Moreover, such an approach oversimplifies the complex relation between psychiatric diagnoses and the lived experiences of people, potentially dismissing the perspectives and knowledge people bring to discussions about their neurodivergent experiences. Of course, someone in this critical category could maintain a strict division between identity and diagnosis. But, again, this amounts to committing to HAP and rejecting, without argument, what we identify as a core demand of the ND movement.

All this shows that many leading criticisms of the ND movement misinterpret some of its claims. If this were the whole story, perhaps the envisioned reconciliation between the ND movement and psychiatry would face only clear sailing ahead. However, we suspect that what truly motivates the critics is something as yet unstated. We suspect that critiques of the ND movement worry about what they see as a potential consequence of capitulating to the movement’s demands, viz., that the movement adopts a certain metaphysics of psychiatric categories in the name of social justice without sufficient support. As we argue in the next section, rejecting HAP does not imply the feared consequence, once more clearing the way to a reconciliation.

4 What really motivates the critics?

As a social phenomenon, the ND movement has political demands: that neurodiversity be recognized, accepted, and not stigmatized in relation to an alleged normality. Again, this faintly echoes some of the claims of the anti-psychiatry movement of the 1960s and 1970s. Both movements are critical of psychiatric practices that strip people of their agency and are exercises of social control, such as coercive treatment or any attempts to cure people who are neurodivergent (Foucault, 1961). Additionally, there are metaphysical claims also shared with the anti-psychiatry movement: that neurodivergences represent another way of experiencing reality rather than being diseases, and that neurotypicality is a normative concept.

对多样需求的认可来促进包容。无论一个人是否承认孤独谱系中‘功能’水平的差异，这一点都可能成立。没有任何事物能阻止医疗援助对那些将其诊断视为部分身份构成的人有所帮助。

正如我们上文所述，那些主张（4）并有动机为精神病学辩护以抵御基于社会不公批评的人，并不需要直接关注神经多样性运动。尽管如此，我们在此发现了这样一种观点：就某些被诊断出的精神病学状况而言，其承载者的认知资质被削弱是这些状况本质的一部分。因此，那种认为精神病学在制造认知不正义的普遍担忧是没有根据的，因为按照这种思路，当一个人的认知能力本身已被削弱时，降低其认知资质并非不正义。虽然Kious等并未明确承诺神经特殊性符合HAP，但他们的论述使其处于一个若拒绝HAP就会陷入困境的立场。他们暗示，某些精神病学分类意味着适用于该分类的人的认知资质被降级。如果我们将其与拒绝相关分类的HAP相结合，就会得出一个有问题的结论：一个人实践或社会身份的某些方面，为拥有该身份的人提供了认知资质被降级的理由。此外，这种方法过度简化了精神病学诊断与人们生活体验之间的复杂关系，可能忽视人们在讨论其神经殊异体验时所提供的视角和知识。当然，处于这一批评范畴的人可以坚持在身份与诊断之间划出严格的界限。但这再次相当于承诺HAP，并在没有论证的情况下，拒绝我们所认定的神经多样性运动的一项核心诉求。

所有这些都表明，许多对神经多样性运动的主要批评误解了其部分主张。如果事情仅此而已，那么神经多样性运动与精神病学之间所设想的和解或许将一帆风顺。然而，我们怀疑，真正驱动批评者的是一些尚未言明的东西。我们怀疑，对神经多样性运动的批评担忧的是，他们看到屈从于该运动的要求可能带来一个后果，即该运动以社会正义的名义，采纳了某种关于精神病学分类的形而上学，却缺乏足够的支持。正如我们在下一节所论证的，拒绝HAP并不意味着会出现令人担忧的后果，这再次为和解扫清了道路。

4 真正驱动批评者的是什么？

作为一种社会现象，神经多样性运动有其政治诉求：即神经多样性应得到承认、接纳，且不应因其与所谓‘正常’的关系而受到污名化。同样，这隐约呼应了20世纪60、70年代反精神病学运动的一些主张。两个运动都批判那些剥夺个人能动性、作为社会控制手段的精神病学实践，例如强制治疗或任何试图‘治愈’神经殊异人士的尝试（福柯，1961）。此外，它们也与反精神病学运动共享一些形而上学主张：即神经特殊性代表了体验现实的另一种方式，而非疾病；神经典型性则是一个规范性概念。

Anti-psychiatry had a hybrid political-theoretical claim at its core: in some of its most strident expressions, it claimed a strong form of anti-realism about psychiatric categories followed from turning a hermeneutical lens towards the structures of power or other social factors that gave rise to their articulation (Szasz, 1960). In this version, anti-psychiatry conceived the biomedical model of mental illness as wrongly applying the principles of medicine to the human psyche. The human psyche, on this showing, is not the sort of thing that can ‘go wrong’ or be dysfunctional in the way that a pancreas or a circulatory system can. This, in turn, was meant to support various *practically* anti-psychiatric attitudes, actions, and social forms. However, as we have already seen, there are important differences between the ND movement and anti-psychiatry, and the ND movement does not commit to these claims.

We suspect that the reason critics of the ND movement are so quick to misinterpret it is because of the great temptation to attribute to the movement the view that, as strong anti-psychiatry held for psychopathology generally, neurodivergences in particular simply elude the sciences of the mind, including psychiatry. This stance would represent a threat to the field of psychiatry, and consequently, it has been thought, to the resources allocated to people in psychiatric need who depend on it. The assumption from critics, we believe, is that ND activists are advancing a controversial claim about the metaphysical status of neurodivergences in the absence of empirical support. According to critics, some ND proponents reduce these categories to neurobiological differences that are, in fact, unknown, and others view them as social constructions that constitute identity. Let us consider these two alleged (and quite opposite) claims in turn.

Forest (2021), a philosopher of science and prominent critic of the ND movement, claims that ND activists use the prefix “neuro” to lend a sheen of scientific legitimacy to their understanding of autism, despite lacking knowledge of neuroscience, in order to legitimize autistic traits. However, Forest argues that the neuroscience of autism is neither sufficient to explain it as a distinct way of being, nor does it align with the social justice demands of ND proponents (Forest, 2021). Russell (2020) also notes that critiques often accuse ND activists of holding reductionist views of the brain.

What do ND proponents have to say about this? First, some explicitly reject it. Nick Walker, for example, claims:

A lot of people hear neuro and they think, brain. But the prefix neuro doesn't mean brain, it means nerve. The neuro in neurodiversity is most usefully understood as a convenient shorthand for the functionality of the whole bodymind and the way the nervous system weaves together cognition and embodiment. So neurodiversity refers to the diversity among minds, or among bodyminds. (Walker and Raymaker, 2020, p.6)

Secondly, the ND movement has embraced an identity that manifests in a bona fide ND culture. According to Chapman (2020), being autistic involves identifying with the social category of autism, which in itself gives the category its importance and legitimacy, even in the absence of scientific validation. This expression of ND culture is sometimes rather described as “a form of agency produced through processes of disidentification with mainstream cultural norms rather than identification with a par-

反精神病学核心是一种混合的政治-理论主张：在其一些最激烈的表述中，它声称通过对催生这些分类表述的权力结构或其他社会因素施加解释学视角，可以得出关于精神病学分类的强烈反实在论形式 (Szasz, 1960)。在这一版本中，反精神病学认为精神疾病的生物医学模型错误地将医学原则应用于人类心灵。根据这种观点，人类心灵并非那种会像胰腺或循环系统那样‘出故障’或功能失调的事物。这进而旨在支持各种实践上反精神病学的态度、行动和社会形态。然而，正如我们已经看到的，神经多样性运动与反精神病学之间存在重要差异，且神经多样性运动并不承诺接受这些主张。

我们怀疑，神经多样性运动的批评者之所以如此急于误解它，是因为一种巨大的诱惑——他们倾向于将以下观点归咎于该运动：正如强反精神病学对精神病理学整体的看法一样，神经殊异性尤其完全超越了包括精神病学在内的心智科学范畴。这种立场将对精神病学领域构成威胁，因此也被认为会危及依赖该领域的、有精神需求者所能获得的资源分配。我们认为，批评者的假设是，神经多样性活动家正在缺乏实证支持的情况下，提出关于神经殊异性形而上学地位的争议性主张。根据批评者的说法，一些神经多样性支持者将这些类别归结为实际上未知的神经生物学差异，而另一些人则将其视为构成身份的社会建构。让我们依次审视这两种被指控的（且截然相反的）主张。

Forest (2021)，一位科学哲学家和神经多样性运动的著名批评者，声称神经多样性活动家使用“神经”这一前缀，是为了给他们对孤独谱系的理解披上一层科学合法性的外衣，尽管他们缺乏神经科学知识，目的是使孤独特质合法化。然而，Forest 认为，关于孤独谱系的神经科学既不足以将其解释为一种独特的生存方式，也不符合神经多样性支持者的社会正义诉求 (Forest, 2021)。Russell (2020) 也指出，批评者常常指责神经多样性活动家持有对大脑的还原论观点。

神经多样性支持者对此有何回应？首先，一些人明确反对这种指责。例如，Nick Walker 声称：

很多人听到神经这个词，就会想到大脑。但前缀“神经”并不意味着大脑，它指的是神经。神经多样性中的神经，最有效的理解是将其视为一个方便的简写，指代整个身心整体的功能以及神经系统如何将认知与具身化交织在一起。因此，神经多样性指的是心智之间，或身心整体之间的多样性。(Walker 和 Raymaker, 2020, p.6)

其次，神经多样性运动所拥抱的身份认同，催生了一种真正的神经多样性文化。根据 Chapman (2020)，作为孤独谱系者，意味着认同孤独谱系这一社会范畴，而这一认同本身便赋予了该范畴重要性和合法性，即使缺乏科学验证。这种神经多样性文化的表达有时更被描述为“一种通过与主流文化规范脱离认同的过程所产生的能动性形式，而非对特定社会范-

ticular social category” (Beck, 2023). In both cases, neurodivergent activism would be considered a form of identity politics. This adherence, and the historical roots of the social model of disability are mistaken for the claim that autism is entirely a social construction (Casanova and Widman, 2021, p. 375) perhaps because its legitimacy comes from ethical and political considerations. But Chapman rejects this idea, claiming instead that the reality of autism is grounded in the lived experiences and social identity of autistic people. On this way of going, autism is real in a way that outstrips *mere* social construction by being a “serial collective,” i.e., the sort of thing that can be “defined in light of shared external material factors that mutually affect each member of the collective, regardless of whether they actually identify or not” (Chapman, 2020, p.810).

Reductionists and social constructionists can be found among the ND movement and its critics alike. Why insist on rejecting a social model of disability while still supporting inclusion and destigmatization, as those advancing (1), above, do? The answer lies in the idea that if the difficulties faced by autistic people were recognized as being solely due to social factors, medical care would become unavailable to those who need it. As Casanova and Widman (2021) claim:

proponents [of the ND movement] oppose potentially useful medical interventions – as not all disability in autistic peoples’ lives necessarily stems from the social environment and a lack of accommodations (p.376).

In contrast, the medical model, by viewing psychiatric categories as designating pathologies that affect individuals, aims to directly address people’s suffering through therapeutic interventions, rather than working on inclusivity and acceptance. This approach is more straightforward when the psychiatric category thought to be inherently harmful is seen as separable from the person’s identity. If this is correct, then the real motivation behind such criticism is HAP. Therefore, the underlying disagreement, we believe, is not about the scientific reality of neurodivergences, but rather about the acceptance or rejection of HAP, which is mistakenly framed as a dispute about the scientific or biological reality of neurodivergences. ND activists are often admirably clear that they do not rest their case on any particular empirical understanding of autism. For instance:

[W]e are concerned with providing a new theorisation of conditions that are understood as impacting on the individual’s sense of identity, alongside differences from standard forms of perceiving and responding to the world. This contrasts with the search to find neurological differences that are perceived as an ‘interruption’ (permanent or otherwise) from an individual’s proper life. Whether the medical conditions coincide with these ‘subjectively experienced’ differences and identities is a matter of empirical investigation. (Rosqvist et al., 2020, p.2).

The theorists who advance criticism (1) appear to be positively asserting the biological reality of neurodivergences against an apparent opponent who denies what they assert. But it is simply not necessary for those who adopt a social model of disability

畴的认同” (Beck, 2023)。在这两种情况下，神经殊异行动主义都可被视为一种身份政治。这种坚持，以及残障社会模式的历史根源，常被误读为声称孤独谱系完全是一种社会建构 (Casanova and Widman, 2021, p. 375)，或许是因为其合法性源于伦理和政治考量。但 Chapman 反对这一观点，转而声称孤独谱系的现实性根植于孤独谱系群体的生活经验和社会身份。按照这种思路，孤独谱系是真实的，其真实性超越了单纯的社会建构，因为它是一个“序列集体”，即那种“可以根据共享的外部物质因素来定义的事物，这些因素相互影响着集体中的每个成员，无论他们是否实际认同该集体” (Chapman, 2020, p.810)。

神经多样性运动及其批评者中都可见到还原论者和社会建构论者。为何像前文 (1) 的支持者那样，既支持包容和去污名化，却又坚持拒绝残障社会模式？答案在于这样一种观点：如果孤独谱系群体面临的困难被认定为完全由社会因素造成，那么需要医疗护理的人将无法获得它。正如 Casanova 和 Widman (2021) 所言：

神经多样性运动的支持者[] 反对可能有益的医学干预——因为并非孤独谱系群体生活中的所有残障都必然源于社会环境及缺乏便利条件 (第376页)。

相比之下，医学模型将精神病学分类视为标示影响个体的病理状态，旨在通过治疗干预直接解决人们的痛苦，而非致力于包容与接纳。当被认为本质有害的精神病学分类被视为可与个人身份分离时，这种方法更为直接。如果这是正确的，那么此类批评背后的真实动机是 HAP。因此，我们认为，根本的分歧并非关于神经殊异性的科学现实，而是关于接受或拒绝 HAP，这被错误地框定为一场关于神经殊异性科学或生物学现实的争论。神经多样性活动家们常常明确地表示，他们的主张并不依赖于任何特定的关于孤独谱系的实证理解，这一点令人钦佩。例如：

[我们关注的是，对那些被理解为影响个人身份感的条件提供一种新的理论化阐述，同时关注其与-感知和回应世界的标准形式之间的差异。这与那种寻找被视为对个人正常生活的“打断”（永久性的或其他的）的神经学差异的探索形成对比。医学状况是否与这些“主观体验到的”差异和身份认同相吻合，是一个需要实证研究的问题。 (Rosqvist et al., 2020, p.2).

提出批评 (1) 的理论家们似乎在积极地主张神经殊异性的生物学真实性，以对抗一个看似否认他们主张的对手。但对于那些采纳残障社会模式的人来说，这根本没有必要。

— and we have seen that not all ND activists do — to also commit to any claim at all about the biological reality of autism.

Those who advance criticism (2), about de-pathologizing, appear to be pleading with the ND movement to recognize that if treatment of autism could be grounded in the biomedical model, the lives of autistic people would be improved by increased access to therapeutic care. As we have seen, it is understandable that HAP is often seen as an expediting assumption which facilitates successful therapeutic encounters. Something similar can be said about proponents of (3). But these are solely pragmatic considerations. Between the medical and social models of disability, it is the medical model that allows access to therapeutic resources because it presupposes that people are separable from their condition and that it can either be treated or cured. The social model, which many ND proponents follow, must rethink the therapeutic encounter. None of these discussions are based on empirical evidence showing that neurodivergences are inherently biological, and nothing in the social model of disability precludes these categories from having a biological basis.

Seeing this also allows the ND movement to emerge somewhat more fully from the shadow of anti-psychiatry. If we are right that the worried critics are legitimately ready to accommodate the demands of ND movement, this ought to be more welcome news. We think proponents of the ND movement are committed to a defense of psychiatry against the Szaszian idea that psychiatry is necessarily unscientific, harmful, and always relies on categories revealed hermeneutically to be problematically constructed (Chapman, 2023). But we think that we should interpret the ND movement as resting its practical claims on the denial of HAP. So understood, there ought to be no temptation to associate the movement with Szaszian claims. If this is correct, the grounds on which the practical claims rest are not only neutral with respect to the metaphysics of pathology or disorder, they are also not motivated by anti-psychiatry-style assumptions.

5 Conclusion

We hope to have shed light on what the disagreement between ND activists and their critics is really about. We have argued that underlying the dispute between the ND movement and its critics is an assumption of HAP by the critics or a failure to recognize that the ND movement rejects it.

Having clarified the nature of the dispute, we think it is now easier to see what needs to happen to enable a successful reconciliation between the ND movement and the psychiatric establishment. For instance, consider the call by some scholars, especially those who identify as neurodiverse and engage in reflections on epistemic authority about neurodivergences (Catala, 2023), for the active inclusion of first-person perspectives in research on neurodiverse conditions. Such work aspires to build the envisioned reconciliation directly into research methodology. If successful, it could function as a kind of proof-of-concept. In this connection, we may also consider, e.g., empirical work aiming to show the positive effects of self-identification with neurodivergence (Kroll et al., 2024).

——而且我们已经看到，并非所有神经多样性活动家都如此——还需要对孤独谱系的生物学现实性做出任何断言。

提出批评 (2) ——关于去病理化——的人，似乎在恳请神经多样性运动认识到：如果对孤独谱系的治疗能基于生物医学模型，那么通过增加治疗护理的可及性，孤独谱系群体的生活将得到改善。正如我们所见，HAP常被视为一种促进成功治疗接触的权宜性假设，这是可以理解的。对于 (3) 的支持者，也可以说类似的话。但这些都仅仅是实用主义的考量。在残障的医学模型与社会模型之间，正是医学模型允许人们获得治疗资源，因为它预设了人与其状况是可分离的，并且该状况可以被治疗或治愈。而许多神经多样性支持者所遵循的社会模型，则必须重新思考治疗接触。所有这些讨论都不是基于表明神经特殊性本质上是生物学的经验证据，并且残障社会模式中的任何内容都不排除这些类别具有生物学基础。

认识到这一点，也使得神经多样性运动能更彻底地从反精神病学的阴影中显现出来。如果我们关于忧心忡忡的批评者们已准备好接纳神经多样性运动诉求的看法是正确的，那么这应该是更受欢迎的消息。我们认为，神经多样性运动的支持者致力于为精神病学辩护，以反驳萨斯式观点——即精神病学必然是不科学的、有害的，并且总是依赖于那些通过诠释学揭示出存在建构问题的范畴 (Chapman, 2023)。但我们认为，应该将神经多样性运动解读为将其实践主张建立在否定HAP的基础上。如此理解，就不应有将该运动与萨斯式主张联系起来的倾向。如果这是正确的，那么其实践主张所依据的根基，不仅在对病理学或障碍的形而上学层面保持中立，而且其动机也并非源于反精神病学的预设。

5 结论

我们希望阐明神经多样性活动家与其批评者之间分歧的真正所在。我们认为，神经多样性运动与其批评者之间的争议，其根源在于批评者预设了HAP，或未能认识到神经多样性运动拒绝接受HAP。

在厘清争议的性质后，我们认为现在更容易看清，要实现神经多样性运动与精神病学体系之间的成功和解，需要做些什么。例如，考虑一些学者，尤其是那些自认为神经多元并参与认知权威反思的学者 (Catala, 2023)，所提出的积极纳入第一人称视角来研究神经多元状况的呼吁。此类工作旨在将所设想和解直接融入研究方法论中。如果成功，它可以作为一种概念验证。在这方面，我们也可以考虑，旨在展示自我认同神经特殊性带来积极影响的实证研究 (Kroll 等人, 2024)。

Whether a successful reconciliation can be effected remains to be seen, but as we have argued, many of the seeming barriers to it are founded on misapprehensions: contrary to the claims attributed to the ND movement by its critics, the movement does not equate the rejection of cure with a rejection of therapy; the ND movement's demand is not an affirmation of a particular claim about the scientific status of neurodivergences or an adherence to an underlying theory of pathology; it does not deny the complexities or the potential biological aspects of neurodivergences. Moreover, ND's stance is not a dismissal of psychiatry's role, but a call for a more nuanced understanding of its application, one that does not aim to "remove" but to provide support. Perhaps most importantly, any worries to the effect that accommodating the ND movement would undermine the scientific credentials of psychiatry are unfounded.

Funding The authors did not receive support from any organization for the submitted work.

Declaration

Competing interests The authors declare no competing interests.

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能否成功实现和解仍有待观察，但正如我们所论证的，许多看似存在的障碍是基于误解：与批评者归咎于神经多样性运动的说法相反，该运动并未将拒绝治愈等同于拒绝治疗；神经多样性运动的要求并非是对神经特殊性科学地位的特定主张的肯定，也非对某种病理学基础理论的坚持；它并不否认神经特殊性的复杂性或潜在的生物学方面。此外，神经多样性运动的立场并非否定精神病学的作用，而是呼吁对其应用有更细致的理解，这种应用的目标不是“消除”，而是提供支持。或许最重要的是，任何关于接纳神经多样性运动会损害精神病学科学信誉的担忧都是没有根据的。

资助声明 作者未从任何组织获得针对所提交工作的支持。

声明

利益竞争 作者声明不存在利益竞争。

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